

Skills

Inserting a Nasogastric Tube

Checklist

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S = Satisfactory U = Unsatisfactory NP = Not Performed

Step	S	U	NP	Comments
Verified the health care provider's orders.	☐	☐	☐	
Performed hand hygiene. Donned appropriate personal protective equipment (PPE) based on the patient's need for isolation precautions or the risk of exposure to bodily fluids.	☐	☐	☐	
Gathered the necessary equipment and supplies.	☐	☐	☐	
Provided for the patient's privacy.	☐	☐	☐	
Introduced self to the patient and family.	☐	☐	☐	
Identified the patient using two identifiers.	☐	☐	☐	
Explained the procedure to the patient and ensured that he or she agreed to treatment. Assessed the patient's cough and gag reflexes.	☐	☐	☐	
Positioned the patient upright in the high Fowler's position, unless contraindicated. When the patient was comatose, raised the head of the bed into the semi-Fowler's position. When the patient must lie supine, placed him or her in the reverse Trendelenburg position.	☐	☐	☐	
Applied the pulse oximeter, and measured the patient's vital signs.	☐	☐	☐	
Assessed the patency of each nare.	☐	☐	☐	
Measured the distance from the tip of the nose to the earlobe, and then from the earlobe to the xiphoid process of the sternum. Added 15.2 cm to ensure gastric placement. Marked the required tube length.	☐	☐	☐	

When the tube had a surface lubricant, dipped it into a glass of room-temperature water. For other tubes, applied a water-soluble lubricant.

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Applied clean gloves.

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Prepared the NG or nasogastric tube for intubation:

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1. Used a 30-mL to 60-mL catheter-tip syringe, injected 10 mL of water into the tube.

Explained the procedure to the patient, and then gently inserted the tube through one nostril to the back of the throat.

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As the tube passed the nasopharynx, had the patient bend his or her head toward the chest.

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Advanced tube along floor of nasal passage, aiming down toward the patient's ear. If resistance was felt, pulled the tube tip back slightly, angled it downward to find the opening to the nasopharynx, and then advanced the tube. If resistance continued, tried to rotating the tube before advancing it. If resistance persisted, withdrew the tube, allowed the patient to rest, bent the tube slightly to angle down when inserted, then lubricated the tube again, and inserted it into the other naris.

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If drinking water was not contraindicated and the patient preferred, provided small sips of water along with encouragement to swallow as the tube is inserted and advanced. Advanced the tube as the patient swallowed. With or without water, emphasized that the patient needed to mouth breathe and swallow throughout the procedure.

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When the tip of the tube was approximately 25 cm to 30 cm, assessed for air coming out of the tubing. If air was present, withdrew the tube and started again. If there was no air, continued to advance the tube to the distance marker.

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Checked the tube position at the back of the patient's throat with a penlight and tongue blade. Ensured tube was not coiled in posterior pharynx.

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Anchored tube to patient's cheek with tape.

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Verified placement of nasogastric tube per organization's practice.

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Clamped the tube.

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Secured tube to the patient's face. Anchored the tube to the patient's nose, avoiding pressure on the nares. Marked the exit site on the tube with tape or indelible ink.

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Labeled tubing at a site close to patient and at a site close to source when there are different access sites or several bags.

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Fastened the end of the NG tube to the patient's gown using a clip or a piece of tape.

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Helped the patient into a comfortable position.

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Obtained a chest or abdominal x-ray to verify placement.

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Applied clean gloves, and administered oral hygiene. Cleaned the tubing at the nostril with a washcloth dampened in mild soap and water.

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Disposed of used supplies.

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Removed and disposed of used gloves. Performed hand hygiene.

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Helped the patient into a comfortable position, and placed toiletries and personal items within reach.

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Placed the call light within easy reach, and made sure the patient knew how to use it to summon assistance.

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Raised the appropriate number of side rails and lowered the bed to the lowest position.

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Learner: _____
Evaluator: _____
Date: _____

Signature: _____
Signature: _____